FREE FAMILY EDUCATION · GUIDE 04

The First 14 Days

Your Newborn Care Companion

A calm companion for everyday newborn observations, safe sleep planning, care routines, and questions for the pediatric team.

Printable workbook · US Letter

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A new rhythm takes time

The first days can bring tenderness, learning, and interrupted rest. There is no single correct rhythm. Keep your baby's discharge instructions and clinician's contact information close; this workbook is for observation and planning, not diagnosis.

When you are concerned

Contact your baby's clinician promptly if feeding changes, your baby is difficult to wake, you notice fewer wet diapers than expected, or something feels wrong. For breathing trouble, blue or gray color, unresponsiveness, or another emergency, call emergency services.

A NOTE FROM TEESEE

A Note from Teesee: Small acts of care add up. Let trusted people take practical tasks while you learn your baby's cues.

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Feeding observations

Follow the feeding plan from your baby's clinician. Feeding patterns vary with age, birth history, and health. Record what you observe rather than trying to interpret it alone.

Feeding observation log

Date/time	Feeding method and duration if advised
Cue or behavior observed	Questions or concern
Follow-up	

Share promptly with the clinician

- Difficulty waking for feeds
- A sudden change in feeding
- Concern about swallowing or keeping feeds down
- Any feeding plan that is not working for your family

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Diapers and daily patterns

Wet and soiled diaper patterns change over the early days. Your clinician can tell you what to expect for your baby's age and health history. Use this tracker to bring clear observations to a visit.

Diaper tracker

Date/time	Wet
Stool	Color/notes if requested
Question for clinician	

No single tracker diagnoses a problem

If you are worried about output, hydration, jaundice, or your baby's condition, contact the pediatric clinician rather than waiting to complete the chart.

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Safe sleep, every sleep

AAP guidance recommends placing babies on their backs for every sleep on a firm, flat, non-inclined sleep surface designed for infants. Keep pillows, loose blankets, toys, and other soft objects out of the sleep space. Room-share without bed-sharing; return baby to their own sleep surface after feeding or comforting.

Checklist

- ☐ Use a safety-approved crib, bassinet, or play yard with a firm, flat mattress and fitted sheet
- ☐ Place baby on the back for naps and nighttime sleep
- ☐ Keep the sleep space free of loose bedding and soft objects
- ☐ Avoid overheating and follow current pediatric guidance
- ☐ Move a sleeping baby from a sitting device to a firm, flat sleep surface when practical; car seats are for travel
- ☐ Share a room, not a sleep surface, when possible

> **Set up the sleep space before you need it**
>
> Follow current AAP guidance and product instructions. Ask your pediatric clinician about questions specific to your baby.

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Hygiene, soothing, and bonding

Everyday care

- Follow discharge instructions for cord and skin care
- Ask how to care for the diaper area and when to call about a rash
- Keep hands clean before feeding and care
- Use clinician guidance for bathing and temperature

Connection

- Talk, sing, and make eye contact when baby is alert
- Hold baby safely and respond to cues
- Try gentle rocking or a calm voice
- Ask another trusted adult to take a turn

Newborns may cry for many reasons. If you feel overwhelmed, place baby on their back in a safe sleep space and call a trusted person for help. Never shake a baby.

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Appointments and urgent concerns

Pediatric questions

Feeding and weight	Diapers or jaundice
Sleep and safe environment	Medicines or supplements
When and where to follow up	After-hours contact

Seek prompt medical advice

- For a baby 3 months or younger, a rectal temperature of 100.4°F (38°C) or higher needs immediate contact with a clinician
- Trouble breathing, blue/gray color, unusual limpness, or difficulty waking
- A sudden feeding change or concern your baby is seriously unwell

> **If severe or life-threatening**
>
> Call emergency services. Do not wait for the next appointment or use this guide to assess an emergency.

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A simple daily care log

Daily newborn care log

Date	Feeds/observations
Diapers/observations	Sleep-space check
Comfort or care given	Questions for clinician

Use only details that help your care team. You do not need to track every moment. Follow your clinician's instructions if they recommend a specific log.

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Care belongs to the whole support team

Caregiver responsibility plan

Care task	Primary person
Backup person	When/how to hand off
Notes	

Checklist

- [ ] Choose a lead for meals and household basics
- [ ] Agree how to communicate a change or concern
- [ ] Plan caregiver rest and safe handoffs
- [ ] Keep emergency and pediatric contacts visible

Reflection

What is one task someone else can take off the new parent's list today?

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The next few days

Ask your pediatric clinician when the next check is due and whom to contact outside office hours. Keep supplies simple and the sleep space clear. Accepting help with food, laundry, or older children can protect time for recovery and connection.

Optional reflection · Psalm 139:13–14

A reference for families who welcome Christian encouragement, offered without assuming a shared belief.

Sources and further reading

Safe Sleep: 9 Ways to Reduce a Baby's Risk of SIDS & Suffocation
American Academy of Pediatrics / HealthyChildren.org

Fever and Your Baby
American Academy of Pediatrics / HealthyChildren.org

Common Conditions in Newborns
American Academy of Pediatrics / HealthyChildren.org

Educational information reviewed against these sources on October 14, 2026. Confirm current guidance with your qualified care team.

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